

Where does the company stand now?

We faced multiple challenges—[accessing] raw materials, scaling up [production], and global logistics. But we learnt a lot and became more efficient.

After the pandemic, many entities globally started exploring vaccine manufacturing. But success in this space requires more than just technology; manufacturing expertise is key.

Post-pandemic, the excess capacity we built had to be repurposed. We refocused on our pre-Covid pipeline—malaria, HPV, and others. The malaria vaccine launched last year is already saving lives in Africa. This year, we are gearing up to provide the HPV vaccine to the Indian government and export to global markets.

We've also been working on vaccines for mosquito-borne diseases—dengue, yellow fever, chikungunya—especially as climate change increases their spread. Over the next two to three years, you'll see these roll out. We're also looking to expand into Europe and the US, markets that we hadn't traditionally served.

▶▶▶

Were you surprised by the rise of the anti-vaccine lobby?

Not really. During Covid, misinformation and misreporting of adverse events led to doubts,

especially on social media. We must continue educating people about how vaccines and immunisation, overall, are safe and vital for disease prevention and a healthier future.

▶▶▶

Post-Covid, there were concerns in India linking sudden deaths to vaccines. Have any credible studies emerged on this?

There's no conclusive evidence linking any vaccine

trials were mandatory for approval, even if global trials were already done. Now, for vaccines tested in top regulatory markets like the US and the EU, the Indian regulator can waive local trials, saving two to three years. Our chikungunya vaccine, developed with (French biotech company) Valneva and already licensed abroad, could benefit from this. At the same time, our dengue vaccine, developed

Chikungunya in under two years; dengue in about three.

▶▶▶

Any new therapeutic areas you're exploring?

Vaccines remain our focus. We're working on TB, and our influenza vaccine which alongside malaria and HIV, remains a global health challenge. We evaluated biosimilars and non-vaccine products but decided against them due to high

WE'RE GOING TO PROBABLY GO FOR A \$1-BILLION CAPITAL RAISE AT THE END OF THIS YEAR VIA A QIP OR BRING IN A FINANCIAL INVESTOR

| 41

to long-term adverse effects. Some deaths were unfortunately sensationalised or completely unrelated medical events. Most studies and courts haven't established a direct link, though investigations continue where appropriate.

▶▶▶

Covid vaccines got fast-track approvals. Has the industry learnt how to move faster without compromising safety?

Yes. India has instituted progressive regulatory reforms. Earlier, local

in-house, is undergoing full trials in India.

▶▶▶

How large is the Indian market for these new vaccines?

Chikungunya will likely be a travel vaccine, like yellow fever or meningitis. Dengue has wider appeal in regions like India, Southeast Asia, and South America. It's not globally needed, but demand in these regions justifies the investment.

▶▶▶

When do you expect them to hit the market?

competition, patent complexities, and different skill requirements.

We aim to build our own IP, like our pneumonia vaccine, and reduce dependence on partnerships. Owning the technology gives us more control and sustainability.

▶▶▶

Developing vaccines is expensive. How does the pricing policy in India impact innovation?

Vaccine giants earn approximately \$40-50 billion annually. We're at just \$1 billion. Developing a vaccine in India can cost